

PUBLIC RESEARCH NOTE

Novo Nordisk: *franchise, or falling knife?* — is the obesity leader cheap, or is Lilly simply winning?

For long-term, index-leaning investors weighing whether an active overweight in Novo makes sense. A full nine-step diligence on Europe's fallen champion: what the ~14× multiple actually prices in — and whether that makes it a bargain or a value trap. I'm showing my work — not giving a call. Data as of 6 July 2026.

- **The market prices Novo for near-zero growth.** A reverse-DCF (my calculation) implies only ~0–3% annual earnings growth for a decade — for a ~21%/yr historical compounder. Run 5,000 times: fair value clears today's price in ~52% of futures if cash flow normalises — ~0% if it doesn't.
- **Quality is elite but turned down across the board in 2025.** Gross margin 84.7%→81.0%, return on capital 88.5%→39.3%, and free cash flow went **negative** in 2024.
- **Lilly is winning the momentum race.** (Eli Lilly — the US pharma giant and Novo's main rival.) ~\$65bn revenue (+45%) vs Novo ~\$47bn; its Zepbound +175% vs Novo's Wegovy +31%; Lilly's drug won the only head-to-head.
- **The key risk:** if the decline is structural (not cyclical), "cheap" is a value trap — history says the loser of a pharma head-to-head rarely recovers the lead.
- **The swing question:** does growth re-accelerate off 2026's trough? Genuinely uncertain — where the whole case is won or lost.

WHAT'S PRICED IN
~0–3% growth /
decade

THIS NOTE
Analysis, not
advice

METHOD
9-step gated DD,
primary-sourced

Key players & terms

New to this sector? Skim these once and the rest reads easily. At heart this is a story of **two companies racing each other** in obesity and diabetes drugs: **Novo Nordisk** and **Eli Lilly**.

Eli Lilly ("Lilly")

The US pharmaceutical giant and Novo's main competitor in diabetes and obesity. The rivalry runs through this whole note.

Semaglutide

Novo's flagship molecule. Sold as **Ozempic** and **Rybelsus** (diabetes) and **Wegovy** (obesity).

CagriSema

Novo's next-generation obesity drug candidate — its main pipeline hope to answer Lilly.

REDEFINE 1 / 2 / 4

The clinical trials of CagriSema. 1 & 2 are the approval trials (both passed); 4 is the head-to-head vs tirzepatide (narrowly lost).

Medicare cut (IRA)

A US law forcing steep price cuts (~71%) on Novo's semaglutide for Medicare patients from 2027.

GLP-1

The class of drugs behind the boom — they mimic a gut hormone that curbs appetite and blood sugar.

Tirzepatide

Lilly's rival molecule. Sold as **Mounjaro** (diabetes) and **Zepbound** (obesity) — it beat Novo's drug head-to-head.

Orforglipron

Lilly's experimental obesity drug in *pill* form — cheaper and easier to scale than injections.

Reverse-DCF

A valuation check that backs out the growth rate the current share price already assumes.

CER

"Constant exchange rates" — growth with currency swings stripped out, so the underlying trend is clearer.

Company snapshot — read this first

A one-page profile of the company: what it is, how it makes money, how big it is, and the tension this note explores. **This section matches the standalone company snapshot; the rest of the note expands the analysis.** Prices are market quotes; market cap, EV and multiples are my own calculation. All figures from Novo's public filings, as of 6 July 2026.

€193–197bn Market cap	~14× / ~10.3× P/E · EV/EBITDA	DKK 309bn FY25 sales	59.6% Obesity vol. share
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What it is, in plain English. Novo Nordisk is a *Danish pharmaceutical company* and a global leader in diabetes and obesity care. Most revenue comes from one family of GLP-1 drugs based on **semaglutide** — Ozempic and Rybelsus (diabetes), Wegovy (obesity) — plus legacy insulin and a small rare-disease arm. It is **vertically integrated and foundation-controlled** (ordinary shareholders cannot force a sale), earning high margins on patent-protected GLP-1 drugs, mostly in the **US (56% of revenue)**.

Identity & market (6 July 2026)		Business mix — FY2025	
Listing	NOVO-B · NVO (ADR 1:1)	BY SEGMENT	
Price NOVO-B / ADR	DKK 324.0 / \$50.43	Diabetes 67% Obesity 27% 6%	
52-wk range (NOVO-B)	224.3 – 464.6	BY GEOGRAPHY	
Market cap / EV	€193–197bn / €205bn	US 56% International 44%	
Control	Foundation ~77% votes / ~28% economics	Still ~⅓ diabetes by revenue; obesity is the growth engine (+31% CER). Dividend yield ~3.5% (payout ~50%).	
CEO	M. Doustdar (since 2025)		

Five-year quality signature — every metric turned down together in 2025 (shaded)

DKK M / %	2021	2022	2023	2024	2025	TREND
Net sales (CAGR 21.7%)	140,800	176,954	232,261	290,403	309,064	
Gross margin	83.2%	83.9%	84.6%	84.7%	81.0%	
Operating margin	41.7%	42.3%	44.2%	44.2%	41.3%	
ROIC (disclosed)	69.0%	73.6%	88.5%	63.9%	39.3%	
Cash conversion (FCF/NP)	61%	103%	82%	-15%	27.6%	

Source: Novo AR 2025 "Five-year overview" + FY 6-Ks; prices are market quotes; multiples computed. Retrieved 6 July 2026.

THE TENSION THE FULL DILIGENCE TACKLES

"Branded-obesity volume leader (59.6%) in a market that grew ~104% in 2025 — yet it lost the only head-to-head to Lilly (25.5% vs 23.0%) and, for the first time, every quality metric turned down together."

The question: a cyclical dent, or a structural decline?

Key products: Ozempic (≈ biggest single product), Rybelsus, Wegovy (injection + a new oral pill, US launch January 2026), legacy insulins, and a pipeline broader than one molecule (CagriSema — filed, but missed the head-to-head; amycretin — a strong next-generation candidate; plus MASH, cardiovascular and rare-disease assets). **What to watch:** US Wegovy weekly Rx share vs Zepbound · capex declining + free cash flow recovering · 2026 vs guidance (−4 to −12% CER) · whether the 2027 Medicare cut spreads to commercial channels. **Key risks:** single-molecule concentration · structural share loss to Lilly · US net-price reset (IRA/340B) · capex stays elevated · foundation control (a patient, through-cycle owner — but limited minority leverage).

1 The business



As the profile above notes, Novo is still *roughly two-thirds diabetes by revenue*, with obesity the growth engine and semaglutide (Ozempic, Rybelsus, Wegovy) the central molecule. Two further points matter here: Diabetes + Obesity together are ~94% of sales, and the US is 56% of revenue – so US pricing is the dominant external variable. And it is foundation-controlled: the Novo Nordisk Foundation holds ~77% of the votes on ~28% of the economics – a patient, through-cycle owner (it can fund the capacity build-out that public-market pressure would punish), though one that limits minority-holder leverage and takeover optionality. Danish foundation ownership has underwritten some of Europe's most durable compounders; the flip side is that shareholder value is one objective among several.

SEGMENT / GEOGRAPHY – FY2025 (DKK M)	SALES	% TOTAL
Diabetes care	207,109	67%
Obesity care	82,347	27%
Rare disease	19,608	6%
US Operations / International	173,166 / 135,898	56% / 44%

Source: Novo Nordisk FY2025 6-K. Retrieved 6 July 2026.

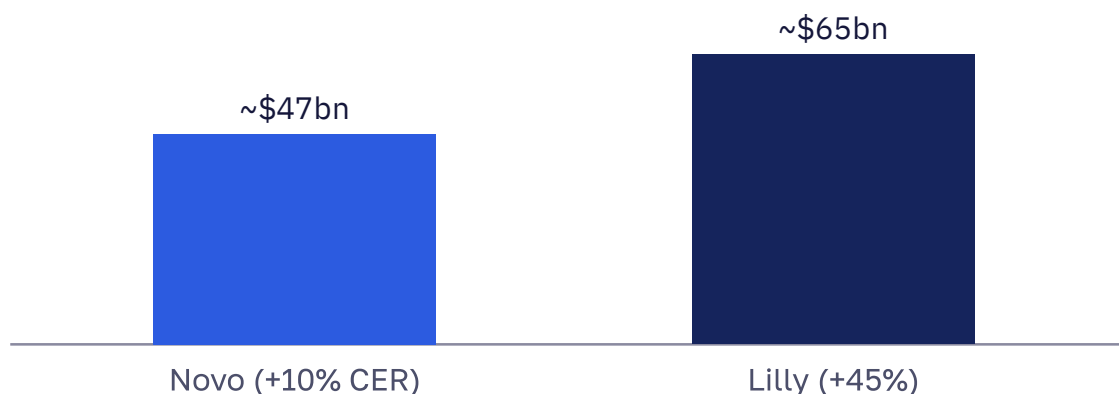
Pipeline – CagriSema, and breadth beyond it

CagriSema – Novo's next-generation obesity drug and its main pipeline hope to answer Lilly – cleared the two trials needed for approval (called **REDEFINE 1 and 2**: 22.7% and 15.7% weight loss vs placebo). What it *lost* was the head-to-head trial (**REDEFINE 4**) against Lilly's rival drug **tirzepatide**, narrowly, on weight loss (23.0% vs 25.5%) – hence "wounded, not dead." (REDEFINE is simply the name of CagriSema's trial programme; the numbers are individual studies within it.) And the pipeline is broader than CagriSema – most notably **amycretin**, a next-generation molecule (in both pill and injection form) whose strong early trial data many read as Novo's real long-term answer to Lilly, alongside candidates in liver disease (MASH), cardiovascular disease and sickle-cell disease – which *partly offsets* Novo's dependence on one molecule.

2 The industry & the duopoly

Branded GLP-1 obesity is a *secular, ~100%-growing market* (up ~104% in 2025) dominated by Novo and Eli Lilly. Novo holds the volume lead (59.6% branded share), but on momentum Lilly is ahead.

Exhibit 1: Lilly is bigger and growing ~5× faster – momentum has shifted



Source: Novo AR 2025 (~\$47bn from DKK 309.1bn, FX-converted); Eli Lilly FY2025 results (SEC / IR, \$65.2bn). Retrieved 6 July 2026.

SCOREBOARD FY2025	NOVO	LILLY
Revenue growth	+10%	+45%
Obesity brand growth	+31%	+175%
Head-to-head weight loss	23.0%	25.5%
2026 revenue guidance	-4 to -12%	~+25%

Source: Novo AR 2025 & Q1 2026 6-K; Eli Lilly FY2025 results.

The oral battle is the decisive axis: Novo's Wegovy pill is a peptide (higher efficacy, harder to scale); Lilly's orforglipron is a small molecule (lower efficacy ~12.4%, but far cheaper to scale) – which could commoditise the tier Novo's pill depends on. **Pricing:** the semaglutide franchise faces a ~71% Medicare price cut from January 2027 (Medicare channel only). The market is secular; Novo's problem is competitive + pricing, not demand – the market grew ~104% while Novo grew ~10%, and that gap is share loss.

"A secular winner's market in which Novo is currently the #2-by-momentum."

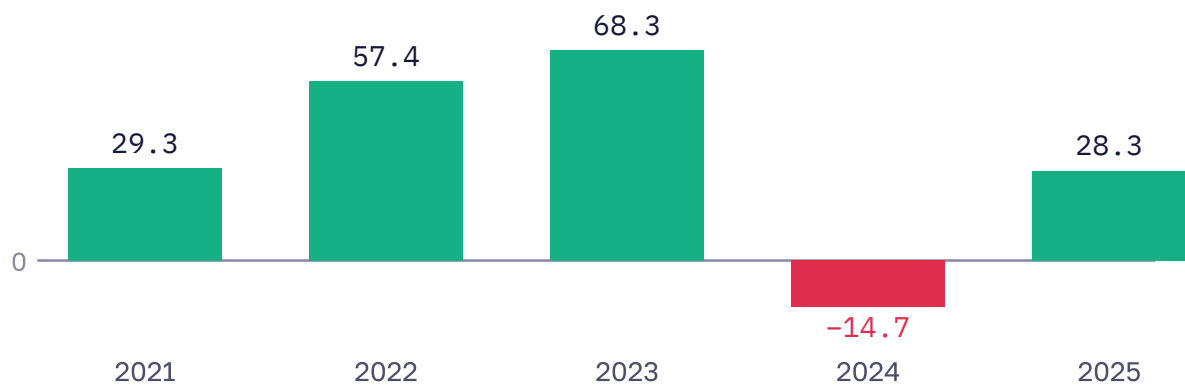
– the cycle / industry lens

3 The financials

DKK M / %	2021	2023	2024	2025
Net sales	140,800	232,261	290,403	309,064
Gross margin	83.2%	84.6%	84.7%	81.0%
ROIC	69.0%	88.5%	63.9%	39.3%
Free cash flow	29,319	68,326	(14,707)	28,295
Share repurchases	19,447	29,924	20,181	1,388

Source: Novo AR 2025 "Five-year overview" + FY 6-Ks. Retrieved 6 July 2026.

Exhibit 2: Free cash flow went negative in 2024 – the valuation needs it to recover

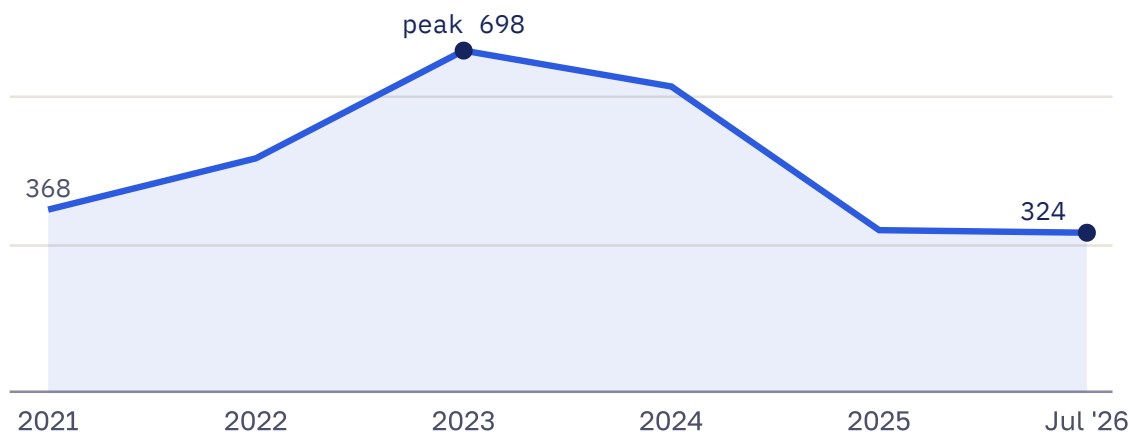


Source: Novo AR 2025 "Five-year overview" (free cash flow, DKK bn; 2024 = -14.7).

The signature is elite by pharma standards but *turned down across the board in 2025*: margin, return on capital and cash conversion all fell as a huge capacity build-out and the Catalent acquisition consumed cash, and buybacks were cut 93% (the dividend was held). The key question is whether that is a cyclical build-out or a structural reset.

4 What's priced in

Exhibit 3: The share is down ~54% from its 2023 peak — the drawdown behind the "ex-growth" price



Source: Novo AR 2025 "Five-year overview" (NOVO-B year-end closing price, DKK) + market quote for July 2026. -54% from the 2023 peak. Retrieved 6 July 2026.

On normalised earnings (assuming capex falls back toward depreciation), today's price implies just *~0–3% annual growth for a decade* at an 8–10% cost of equity — pessimistic for a 21.7% historical compounder. But it's conditional: anchor on today's depressed free cash flow and the same price implies +6–20%, i.e. the price already requires a big recovery. So "cheap" holds only if the capex build-out normalises.

IMPLIED ANNUAL GROWTH PRICED IN	8% COE	10% COE
On normalised earnings (capex normalises)	-1% to +1%	+3%
On today's actual free cash flow	+6% to +15%	+8% to +20%

Reverse-DCF (my calculation, two-stage, 2.5% terminal). Trailing P/E ~14×, EV/EBITDA ~10.3×. [Inferred]

Mapping scenarios to that: the market's implied path sits **between a bear case (flat-to-declining) and a base case (~5–8% growth), closer to bear**. So the setup only needs the base case to be roughly right to offer upside — but it has no cushion if the bear case is right.

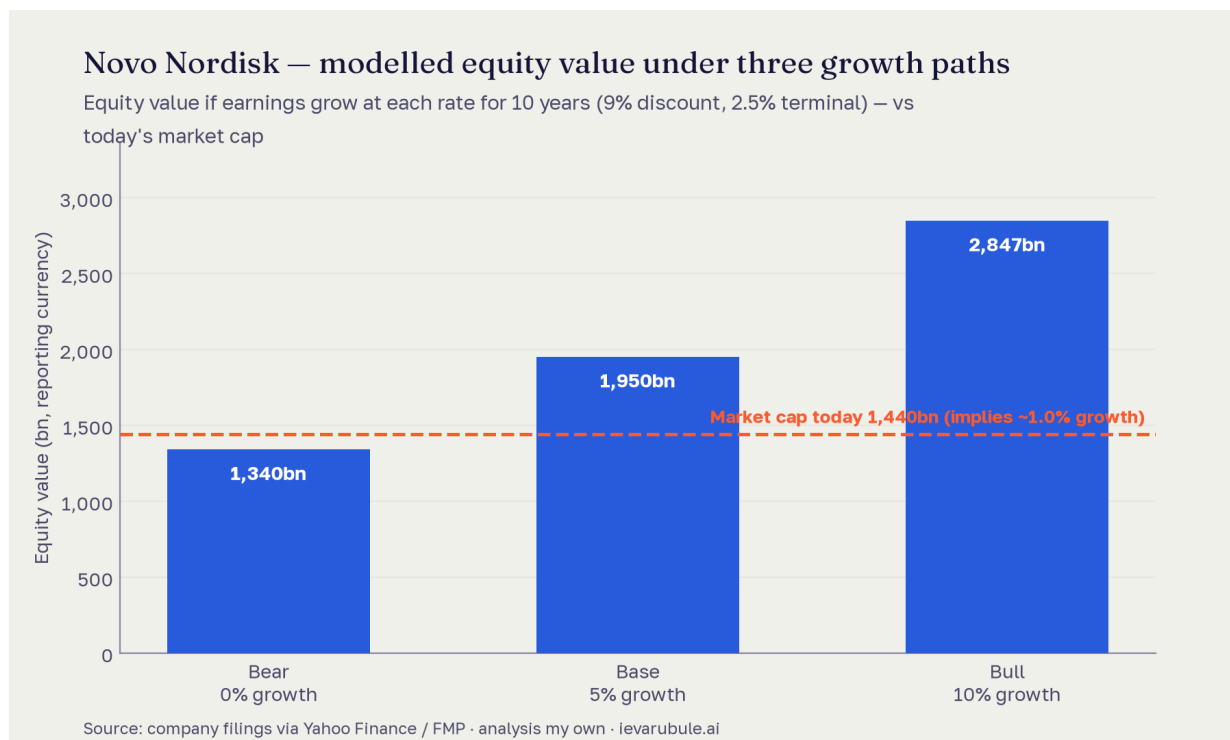
"The price already embeds the bear case — the question, for you to judge, is whether the bear case is right."

— the valuation lens

What the price is assuming – the valuation, run properly

The most useful way to read DKK 324 is backwards. A reverse-DCF asks: what growth does today's price already *require*? The answer: roughly *1% profit growth a year, for a decade* – an ex-growth price for a company that grew north of 20% a year through the GLP-1 boom.

Exhibit 4: What the price is paying for – today's market cap (dashed) sits just above the zero-growth path

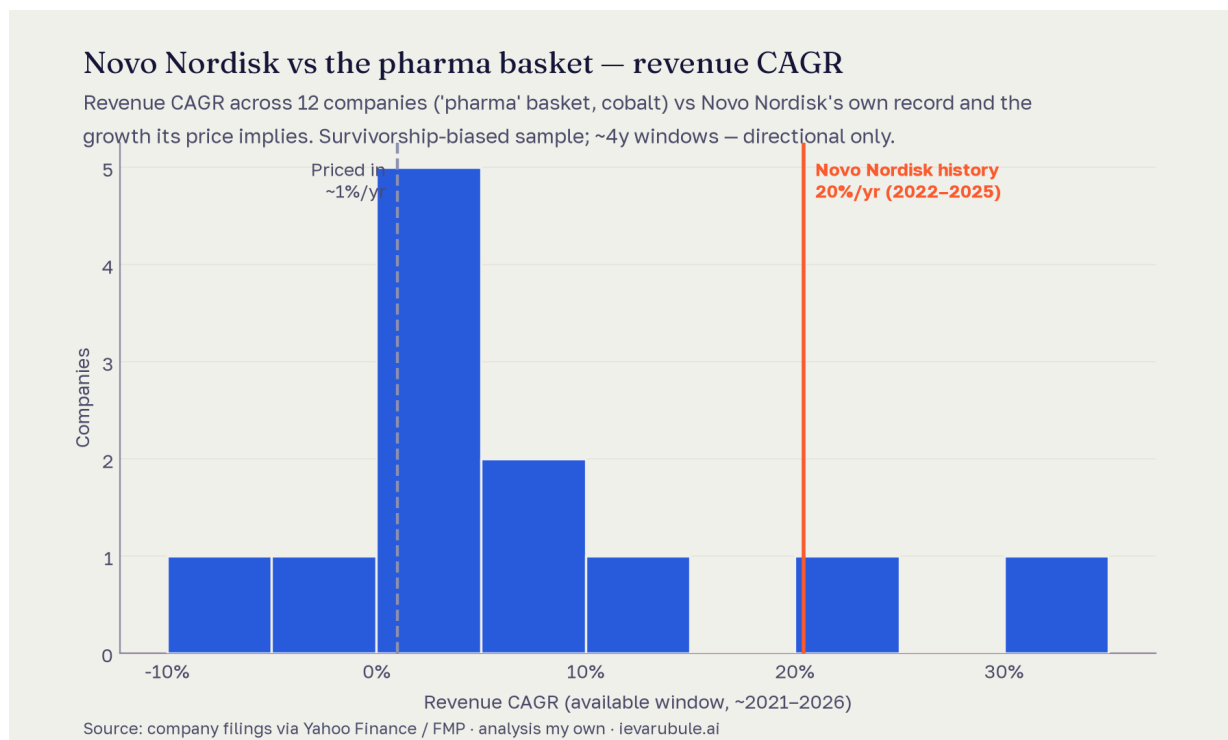


Source: Novo filings + market quotes; my calculation (scenario model, 9% discount, 2.5% terminal). Chart data ships as a CSV twin with this note. [Inferred]

Is ~1% a reasonable expectation? Against eleven actual pharma peers – Lilly, Roche, Novartis, AstraZeneca, Sanofi, GSK, Merck, Pfizer, AbbVie, Amgen, Bristol Myers – two things stand out. The group's median revenue growth was *~4% a year*, and only a quarter sustained more than 10%. But Novo's own record was the *second fastest of the twelve* – while the ~1% now baked into its price would rank *tenth*.

The outside view: what companies like this actually did

Exhibit 5: The peer evidence — Novo's record sits second from the top; the growth its price implies sits near the bottom



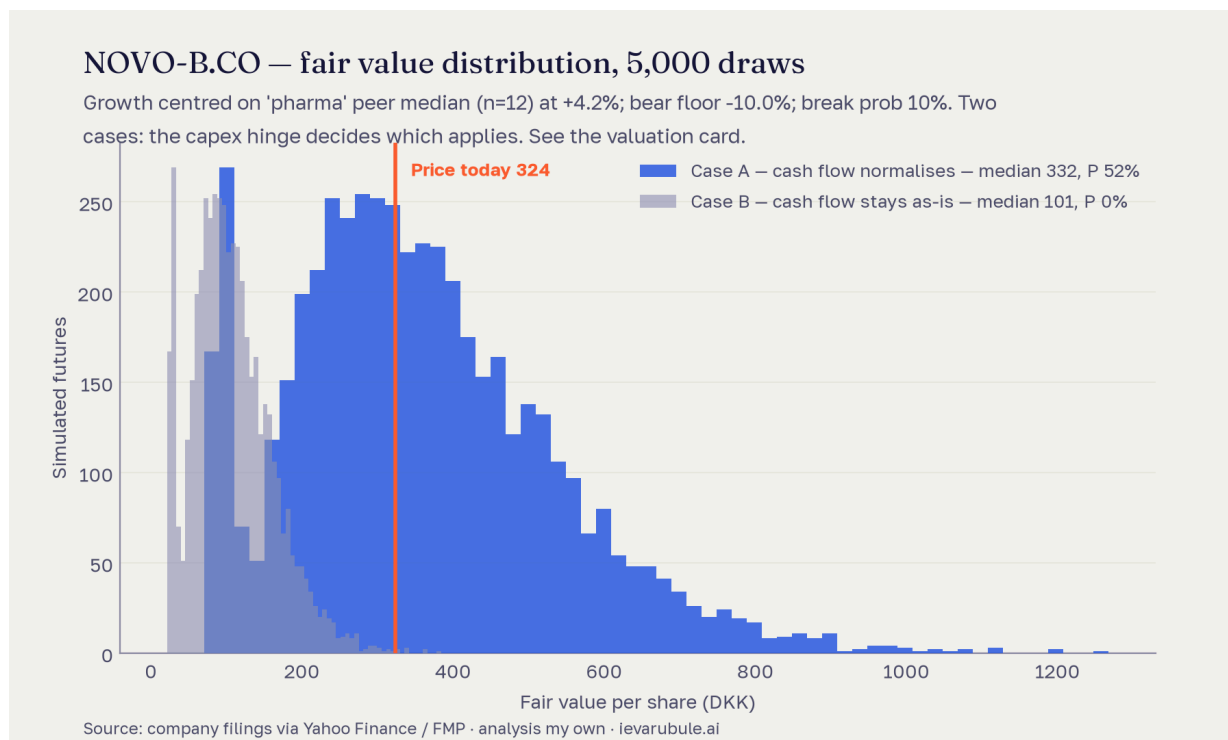
Source: peer company filings via public market data (~4-year windows); my calculation. Survivorship caveat: failed companies left the sample, so true base rates are lower than shown. [Inferred]

In the space of roughly a year, the market has repriced the industry's number-two grower as a near-bottom decliner. That repricing is either right — Lilly winning the head-to-head plus the 2027 Medicare cut really do make Novo a near-bottom grower — or the market is punishing a proven compounder for one difficult stretch. Stories can argue either side; the honest move is to stop arguing and run the distribution.

Five thousand futures, one hinge

So I ran the valuation five thousand times, drawing growth around the *pharma-peer* median – with a real bear floor and a share of outright structural-break scenarios – rather than around hope. The result splits on a single assumption, and it is not the share price.

Exhibit 6: The whole argument in one picture – the gap between the two humps is the capex assumption



Source: my calculation – seeded Monte Carlo, 5,000 draws per case, both cases on the same seed; earnings bases spot-checked to AR 2025 (filed free cash flow DKK 28,989m, p.84). [Inferred]

CASE	MEDIAN FAIR VALUE	P(FAIR VALUE > DKK 324)
A – cash flow normalises toward profit (as guided)	≈ DKK 332	≈ 52%
B – cash flow stays as-is	≈ DKK 101	≈ 0%

Neither case is ever quoted without the other – the gap IS the finding. [Inferred]

If Novo's currently depressed free cash flow normalises back toward its profit once the build-out ends, fair value clears today's price in about half of plausible futures: *a coin flip, not a bargain*. **If cash flow stays where it is, it clears in none of them.** The share price is not really the question. The capex hinge is.

5 The four questions that decide it

QUESTION	WHERE THE EVIDENCE POINTS
Moat = molecule (losing) or franchise (durable)?	Franchise > molecule, but eroding at the margin – defending, not extending.
Capex/ROIC – cyclical or structural?	Partial normalisation likely; returns settle below the old peak.
Can oral + pipeline + International offset Lilly + the price cut?	The swing question – genuinely uncertain.
How deep/permanent is the US price reset?	Structural downtrend, gradual not a cliff (Medicare channel).

Through different investing lenses (perspectives, not a scorecard)

Quality: elite signature, but every metric turned down in 2025; the moat is real yet narrowing. **Cycle:** sentiment swung euphoria→despair – a contrarian setup, if the analysis holds. **Valuation:** undemanding, but conditional on the cash-flow recovery. **Inversion / risk:** the sharpest risk is becoming "the leader who loses the head-to-head and doesn't recover"; mitigants are incumbent share, breadth beyond one molecule, and the dividend. *(Perspectives to weigh – not ratings, not a recommendation.)*

6 Base rates & the key risk

The two sides of the debate, in one box (thesis / antithesis):

LENS	BULL	BEAR
Franchise	GLP-1 secular; breadth beyond semaglutide; a strong #2 can still compound	#2 in a duopoly rarely regains leadership; semaglutide dependence
Economics	Returns still high; margins elite; capex normalises → cash flow recovers	Structural capex arms race; returns permanently lower; cash flow never fully recovers
Pricing	The price cut is a manageable drag (Medicare channel)	It spreads to commercial; net price falls faster than volume

And the base rates – reasoning by reference class (illustrative, not a full dataset):

WHEN THIS HAS HAPPENED BEFORE...	...HOW IT USUALLY GOES
A pharma #2 loses the pivotal head-to-head (e.g. Opdivo vs Keytruda in lung cancer)	The loser rarely recovers the lead
A very-high-ROIC company's returns over time	Fade, but often stabilise above cost of capital
A negotiated (IRA-style) price cut	Gradual drag, not an overnight cliff
Buying quality after a ~50% fall on fear	Bimodal – great, or a value trap

THE KEY RISK (HONESTLY STATED)

Novo becomes "the Opdivo of obesity": the molecular arms race, once lost, compounds – Lilly's lead plus its cheap oral take share, the capex arms race proves permanent so free cash flow never recovers, and the 2027 price cut deepens. On today's cash flow the price already needs a recovery, so if the decline is structural the ~14× multiple is a value trap, not a bargain.

What I'm watching (things to monitor, not instructions)

US Wegovy weekly prescription share vs Zepbound (the earliest tell) · whether 2026 lands near the bottom of guidance and 2027 growth inflects · whether capex declines and free cash flow recovers · whether the Medicare price cut spreads to commercial channels.

7 You draw the conclusion

I keep my own buy/sell/hold decision private, deliberately: it depends on *my* circumstances and portfolio, which aren't yours, and a public "call" is both less useful and more fraught than showing you how to reason about it. So here are the questions I think it comes down to – and only your own honest answers to them can settle it for you.

1. **Do you understand this industry well enough** to judge pipeline and pricing risk – or is it honestly "too hard" for you?
2. **Cyclical or structural?** Was 2025–26 a temporary dent (capacity build-out, accounting/pricing noise) or a permanent worsening of the competitive position?
3. **Do you already own this inside a broad index fund?** If so, does an active overweight clearly earn its place versus simply staying indexed?
4. **What specifically would change your mind** – in either direction – and are you watching for it?

By theme: Q1 = competence · Q2 = structure-vs-cycle · Q3 = portfolio context · Q4 = process.
If your honest answer to Q1 (competence) or Q2 (cyclical-vs-structural) is "too hard" or "no", the default answer for most investors is: stay indexed and watch.

My honest summary of the setup: an elite franchise, cheaply priced for a pessimistic path, at a washed-out sentiment low – *and* a faster-growing competitor that just won the head-to-head, with an unfriendly base rate for #2s that lose. A coin-flip you're compensated for, not a bargain you're stealing. Whether that's worth owning is a judgement only you can make for your own situation.

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